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Zimbabwe's Healthcare System: Resilience, Recovery, and Digital Transformation

The Zimbabwe healthcare system has achieved some of Africa's most striking public health outcomes despite severe economic constraints, including reaching the UNAIDS 95-95-95 HIV targets in 2024. This profile examines the country's health infrastructure, village health worker network, growing digital health capacity, and clinical research institutions.

Kapsule Research Team 19 March 2026 10 min read

The Zimbabwe healthcare system is full of contradictions. The country has achieved world-class HIV treatment outcomes while operating under persistent economic pressure, health worker shortages, and infrastructure gaps. With approximately 16 million people, Zimbabwe spends well below the Abuja Declaration target of 15 percent of its national budget on health, yet has built a nationwide community health network, rolled out electronic health records across hundreds of facilities, and developed clinical research institutions that attract international trial sponsors. For

Zimbabwe operates a tiered public health system managed by the **Ministry of Health and Child Care (MOHCC)**. The sector was governed by the **National Health Strategy 2021-2025**, which targeted UHC, outbreak control within two weeks, and stunting reduction to 16 percent. That plan has now expired, and the **National Development Strategy 2 (NDS2) 2026-2030** provides the successor framework, with health embedded as a core pillar of the country's push toward upper-middle-income status. A **National Health Insurance** bill is working through Parliament with a target launch of June 2026. Primary care is delivered through approximately 1,800 clinics and rural health centres. District and provincial hospitals provide secondary care, while six central hospitals handle tertiary referrals.

Key indicators:

- **Population:** approximately 16 million (2025 estimate)
- **Life expectancy at birth:** 62.4 years (2024, Macrotrends)
- **Infant mortality rate:** 33.4 per 1,000 live births (2024, Macrotrends)
- **Maternal mortality ratio:** 212 per 100,000 live births, down from 651 in 2015 (ZDHS)
- **HIV prevalence:** approximately 11 percent among adults aged 15 to 49
- **Health facility ratio:** national average of 1.1 per 10,000 population (most provinces around 1.6), against a national target of 11 per 10,000
- **Health budget allocation:** 9 percent of the national budget in 2024, below the 15 percent Abuja target

Life expectancy has recovered markedly from its nadir of roughly 44 years during the

Healthcare in Zimbabwe is delivered through a four-tier referral system. At the base, rural health centres and clinics provide primary care, immunisation, antenatal services, and basic curative treatment. District hospitals offer surgical and inpatient care. Provincial hospitals provide specialist services, while the central hospitals in Harare and Bulawayo serve as apex referral centres.

The two largest **hospitals in Zimbabwe** are **Parirenyatwa Group of Hospitals**, with approximately 1,800 beds and over 2,000 staff, and **Sally Mugabe Central Hospital** (formerly Harare Central Hospital), with roughly 1,200 beds. Both serve as teaching hospitals affiliated with the University of Zimbabwe's Faculty of Medicine and Health Sciences.

The system faces a severe workforce crisis. Between June 2023 and June 2024, nearly 36,000 Zimbabweans were granted work visas to the United Kingdom, the majority as healthcare workers. Repeated strikes by nurses and doctors over low pay have compounded service delivery gaps, particularly in rural districts. The brain drain to South Africa, Botswana, and the UK has left many facilities operating with skeleton staff.

Health financing relies heavily on external donors. According to World Bank data from 2021, external sources contributed approximately 45 percent of current health expenditure, while domestic public funding covered about 34.7 percent (Zimbabwe National Health Accounts, 2015) and out-of-pocket spending accounted for roughly 10 percent. Per capita health expenditure stood at approximately USD 103 in 2019, among the lowest in the Southern African region.

UNAIDS 95-95-95 Fast-Track targets for adults, one of the first countries in Africa to do so. Of those living with HIV, 95 percent know their status, 99 percent of diagnosed individuals receive antiretroviral therapy, and 96 percent of those on treatment have achieved viral suppression. HIV prevalence peaked at 25 percent in the early 2000s, so these numbers represent a real turnaround.

The HIV programme's success owes much to sustained support from **PEPFAR**, the **Global Fund**, and USAID, alongside strong domestic programme management. Coverage of pregnant women living with HIV receiving antiretrovirals for prevention of mother-to-child transmission reached approximately 94 to 96 percent by 2018, depending on source and indicator definition (UNAIDS, Zimbabwe GARPR). However, continued dependence on external funding creates vulnerability, particularly as donor priorities shift.

Malaria remains endemic in lower-altitude areas. In 2023, Zimbabwe recorded approximately 248,700 confirmed malaria cases, with the country experiencing a significant surge in infections during early 2025. Tuberculosis co-infection with HIV continues to drive TB morbidity, though case detection and treatment completion rates have improved with integrated HIV-TB service delivery.

Non-communicable diseases are a growing concern. Hypertension, diabetes, and cardiovascular disease are rising, driven by urbanisation and changing dietary patterns. The health system's capacity to manage chronic NCDs alongside its infectious disease burden is an emerging strategic challenge.

Village Health Worker Programme

programme aimed to train 15,000 village-based health workers to extend primary care to communities with no access to formal health facilities.

VHWs perform a broad set of functions:

- Health promotion, including hygiene, sanitation, and nutrition education
- Maternal and child health support, including family planning counselling
- Malaria diagnosis using rapid diagnostic tests and treatment of uncomplicated cases
- HIV testing referrals and adherence support for patients on antiretroviral therapy
- Disease surveillance and community-level data collection

The programme has contributed to measurable gains in family planning uptake, declining new HIV infections, and reduced AIDS-related mortality at community level. The **village health workers Zimbabwe** model has influenced community health strategies in several neighbouring countries.

The programme has scaled beyond its original target. As of early 2026, Zimbabwe has approximately 21,000 VHWs nationwide, exceeding the original 15,000 goal, though an estimated 15,000 of those were previously funded by NGOs rather than the government. In February 2026, the government began a phased integration of VHWs into the national payroll, converting NGO-funded allowances into government salaries at roughly three times the previous amount, with full integration targeted by end of 2027.

The headline numbers obscure real problems, though. Geographic coverage remains remuneration, inadequate supervision, and drug supply gaps were the primary drivers of VHW attrition. UNICEF reported training 23,864 VHWs in integrated community case management in 2024, which suggests renewed investment in the cadre's clinical capabilities.

The **National Health Strategy 2021-2025** and the companion **National Community Health Strategy 2020-2025** both prioritise revitalisation of the VHW programme to bridge the gap between the formal health system and communities. Platforms like Kapsule, which aggregate and structure clinical encounter data from community and facility-level sources, can help quantify VHW programme impact and identify coverage gaps through real-time data analysis.

Digital Health and Data Infrastructure

Digital health in Zimbabwe has moved forward under the MOHCC's National Digital Health Strategy 2021-2025. The centrepiece is **Impilo**, a locally developed web-based electronic health record system first piloted in 2016 for HIV patient tracking.

By 2023, Impilo had expanded to 1,055 of Zimbabwe's approximately 1,800 primary and secondary care facilities, covering 18 clinical modules spanning HIV, TB, maternal health, immunisation, and non-communicable disease management. In Matabeleland South Province, 90 percent of health facilities had EHR infrastructure operational as of early 2025.

Zimbabwe is also working to adopt **HL7 FHIR** (Fast Healthcare Interoperability Resources) standards within Impilo, supported by the Global Fund. This interoperability

Challenges persist. Power instability and limited internet connectivity in rural districts disrupt system availability. The MOHCC has deployed adaptive solutions, including WhatsApp-based helpdesks for field support and mobile data backups. Legislative frameworks for eHealth governance and health data protection are still underdeveloped, which will need to be sorted out as digital systems scale.

Zimbabwe is one of the few countries in Southern Africa building a national EHR system from a domestically developed platform, which may offer greater long-term adaptability than imported solutions.

Clinical Research Capacity

Zimbabwe has real clinical trial infrastructure, built around several institutions. The **University of Zimbabwe Clinical Trials Research Centre (UZ-CTRC)**, established in 1994 through a partnership with the University of California San Francisco, has conducted multi-centre paediatric and adult HIV clinical trials for over 30 years. Its research focus spans HIV, women's health, adolescent health, and associated non-communicable diseases.

The **Biomedical Research and Training Institute (BRTI)** is a non-profit research organisation that has held grants from the NIH, Wellcome Trust, and the UK Medical Research Council since the mid-1990s. BRTI's work covers infectious disease epidemiology, genomics, and health systems research, with a particular emphasis on building local research capacity.

Regulatory oversight for clinical trials falls under the **Medicines Control Authority of Zimbabwe (MCAZ)**, established in 1997 as the successor to the Drugs Control Council (founded 1969) under the Medicines and Allied Substances Control Act. MCAZ achieved **WHO Global Benchmarking Maturity Level 3** status in June 2024, which means it meets international regulatory standards. In January 2026, MCAZ published Revision 4 of its Guidelines for Clinical Trial Application and Authorization, streamlining the approval process for sponsors.

The Medical Research Council of Zimbabwe (MRCZ) provides ethical review for all human subjects research conducted in the country. Together, MCAZ and MRCZ provide a regulatory and ethics framework that supports both locally led and internationally sponsored trials.

Opportunities for Health System Recovery and Innovation

Zimbabwe's health system operates under well-documented constraints: fiscal pressure, workforce emigration, infrastructure deficits, and donor dependence. But within those constraints, the country has built real assets for pharma companies, CROs, and global health organisations evaluating Southern African partnerships.

The HIV programme shows Zimbabwe can execute large-scale treatment programmes to international standards. The Impilo EHR rollout shows digital health infrastructure is advancing at facility level. UZ-CTRC and BRTI have decades of trial infrastructure), NCD research as chronic disease burden grows, structured data partnerships as Impilo scales and interoperability standards mature, and community health data from the VHW network in underserved areas.

Zimbabwe has the epidemiological relevance, institutional research depth, and advancing digital infrastructure to warrant real engagement from organisations working across Southern and East Africa.

Kapsule provides access to structured, de-identified health records from over three million patient encounters across East and West Africa, with standing ethics approvals in Rwanda, Kenya, Uganda, Nigeria, and Ghana. **Contact our team** to discuss how Southern African health data can support your clinical development and research strategy.

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